Dermatology Service Line Handbook

MERIDIAN HEALTH PARTNERS ## OPERATIONS POLICY MANUAL: DERMATOLOGY SERVICE LINE Document ID: MHP-POL-DERM-412 Effective Date: January 15, 2026 Applies To: All 14 Meridian Health Partners outpatient clinics Author: Sarah Lin, Operations Policy Lead

1. PURPOSE AND SCOPE This document establishes the operational parameters, scheduling rules, insurance tier mappings, and referral processing standards for the Dermatology service line across all Meridian Health Partners facilities. Compliance with this policy is mandatory for all scheduling coordinators, medical assistants, and practice managers.

2. CONDITIONS ACCEPTED AND EXPLICITLY NOT ACCEPTED 2.1 Accepted Conditions The Dermatology department accepts patients presenting with the following diagnosed or suspected conditions for evaluation and procedural management: * Acne vulgaris (moderate to severe) and rosacea * Atopic dermatitis, contact dermatitis, and localized psoriasis * Benign cutaneous neoplasms, epidermal cysts, and lipomas requiring excision * Atypical pigmented lesions and actinic keratoses * Alopecia areata and scarring alopecias

2.2 Explicitly Not Accepted Conditions
The Dermatology department does *not* accept, evaluate, or schedule appointments for the following clinical presentations. Referrals received for these conditions will be declined within the timeframe specified in Section 5:
* Cutaneous T-cell lymphoma or advanced metastatic melanoma requiring systemic chemotherapy (must be referred directly to Meridian Oncology)
* Severe systemic autoimmune blistering diseases, including pemphigus vulgaris and bullous pemphigoid (must be referred to Meridian Rheumatology)
* Elective cosmetic procedures, including chemical peels, non-ablative laser resurfacing, sclerotherapy for telangiectasias, and cosmetic botulinum toxin injections

3. TYPICAL APPOINTMENT DURATION Scheduling templates must strictly adhere to the following time allocations based on visit type. No provider may overbook standard slots without prior written authorization from Dr. Marcus Vance, Chief Medical Officer. * New Patient Consultations: 40 minutes * Routine Follow-Up Visits: 20 minutes * Excision / Procedural Appointments: 60 minutes * Biopsy-Only Visits: 20 minutes

4. PATIENT PREPARATION INSTRUCTIONS Scheduling staff must deliver the following preparation instructions to patients at the time of booking, supplemented by automated SMS or email dispatch: * Biopsies and Excisions: Patients must discontinue the use of topical retinoids on the target site 72 hours prior to the procedure. Blood thinners (warfarin, direct oral anticoagulants, and daily aspirin taken for primary prevention) must not be

stopped without explicit clearance from the prescribing cardiologist or primary care physician. * Skin-Check Evaluations: Patients must arrive with no facial makeup, body lotions, or colored nail polish, as these impair visual inspection of lesions and nail units. * Patch Testing: Patients must cease all systemic corticosteroids 14 days prior to the application of allergen panels and topical steroids on the back for 7 days prior.

5. REFERRAL TURNAROUND COMMITMENT 5.1 Processing SLA The Dermatology department commits to a strict internal referral turnaround time of 48 business hours from the moment of electronic health record (EHR) intake to patient contact for appointment scheduling.

5.2 Escalation Protocol
If a referral lacks required clinical notes or insurance pre-authorization documentation, the intake specialist must issue a formal request for information (RFI) to the referring provider within 24 business hours. If the RFI is not satisfied within 96 business hours, the referral is administratively closed and returned to the sender.

6. INSURANCE PLAN COVERAGE MATRIX Meridian Health Partners operates four proprietary health plans. Coverage for dermatology procedures is strictly governed by the following tier assignments:

Procedure Category	MeridianCare (HMO)	MeridianChoice (PPO)	MeridianSelect (EPO)	MeridianAssist (Medicaid)
Diagnostic Biopsies	Covered (PCP Referral Req.)	Covered in-network	Covered in-network	Covered (PCP Referral Req.)
Benign Lesion Excision	Covered if symptomatic	Covered	Covered	Covered if symptomatic
Malignant Lesion Excision	Fully Covered	Fully Covered	Fully Covered	Fully Covered
Phototherapy (NBUVB)	Covered (Prior Auth Req.)	Covered	Covered	Covered (Prior Auth Req.)
Cosmetic Procedures	Not Covered	Not Covered	Not Covered	Not Covered

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* Contact for Deviations: Operations Policy Desk (ext. 4411)